

PRE-LECTURE ANALYSIS SHEET (Zhang + Flinner)

Divergent Channels, Sinew Channels & Cutaneous Regions

Jing Bie / Jing Jin / Pi Bu

Prof. (Dr.) Vivian Zhang, Ph.D. + Dr. Justin Flinner (parallel section)

THIS WEEK IS NOT THE EXTRAORDINARY VESSELS.

Per Dr. Zhang's own Week 1 syllabus reading: Week 7 = Divergent Channels, Sinew Channels & Cutaneous Regions. The Eight Extraordinary Vessels are Week 8.

Reading: MOA p.367-370, 387-390, 417-421, 469-472; CAM p.77-82 (per Lecture 6 'For Next Week' slide) · Channels Projects due Week 8

This lecture hasn't happened yet -- this is a PRE-LECTURE draft built from the syllabus + Week 1 preview material only.
BEFORE lecture: Complete Sections A-D. Rate confidence honestly -- low pre-scores are expected for unlectured material.
DURING lecture: Annotate directly on this sheet. Correct anything below that Dr. Zhang teaches differently.
AFTER lecture: Complete Section F within 24 hours, then this sheet becomes your first real Week 7 reference.

A

Learning Targets — I Can Statements

Pre/post confidence ratings 1-5 for 8 learning outcomes

B

Activate — Connect to What You Know

Bridge from Week 1 preview + fill-in prompts

C

Vocabulary Pre-Load

10 key terms: Pinyin, English, space for your definition

D

Anticipatory Questions

11 questions across 4 groups; starred = high-challenge/high-yield

FL

Flinner Lens (supplementary)

Cross-references already-verified Flinner facts -- flagged, not new

IQ

Inter-Quiz: Mid-Study Probe

12 items, 3 checkpoints; mastery ≥ 10/12 x3

E

Clinical Case

Pre-lecture reasoning + post-lecture protocol

F

After-Lecture Synthesis

3-column synthesis + review notes

LEARNING TARGETS -- I CAN STATEMENTS

Purpose: These define what you should be able to DO after lecture, not just recognize.

Pre-lecture note: it's normal for every Pre score to be low/1 here -- this material hasn't been taught yet.

Define a Divergent Channel (Jing Bie) and state how it differs functionally from a primary channel.

Pre-Lecture 1 2 3 4 5

Post 1 2 3 4 5

State where divergent channels mainly distribute (chest, abdomen, head) and their core function -- deepening the interior-exterior (Biao-Li) relationship between paired channels.

Pre-Lecture 1 2 3 4 5

Post 1 2 3 4 5

Name the pattern of a divergent channel's path using the 4 verbs: Li (separate) -> Ru (enter) -> Chu (emerge) -> He (converge).

Pre-Lecture 1 2 3 4 5

Post 1 2 3 4 5

List all 6 Confluences (Liu He) by their paired primary channels.

Pre-Lecture 1 2 3 4 5

Post 1 2 3 4 5

Define a Sinew Channel (Jing Jin) and state the one thing it does NOT have that a primary channel does (no pertaining Zang/Fu organ).

Pre-Lecture 1 2 3 4 5

Post 1 2 3 4 5

Define a Cutaneous Region (Pi Bu) as the surface/skin reflection zone of a primary channel, and state its diagnostic use.

Pre-Lecture 1 2 3 4 5

Post 1 2 3 4 5

Distinguish all 4 supplementary systems from each other in one line each: Collaterals (Luo), Divergent Channels (Jing Bie), Sinew Channels (Jing Jin), Cutaneous Regions (Pi Bu).

Pre-Lecture 1 2 3 4 5

Post 1 2 3 4 5

TRAP: state clearly that this week is NOT the Eight Extraordinary Vessels -- that's Week 8. Don't let study time bleed into next week's material by mistake.

Pre-Lecture 1 2 3 4 5

Post 1 2 3 4 5

Purpose: Activating prior knowledge before new input improves encoding.

B1: Write whatever Dr. Zhang previewed in Week 1 about these 3 systems. B2: fill in before lecture.

B1 . What Dr. Zhang Already Told You (Week 1 preview)

In Week 1 Dr. Zhang previewed these 3 systems briefly: divergent channels 'govern the inside of the body... branched out from the primary meridians and mainly distributed on the chest, abdomen, and head... enhancing the relationship' between paired organs. Sinew channels are 'distribution of the primary meridians into muscles, reflecting the functional relationship between channels and body movement.' Cutaneous regions are 'parts of the 12 meridians reflected on the body surface skin, where the Qi and meridians distribute.'

Write it in your own words -- what do you think each system DOES that primary channels alone don't?

B2 . Connect the Dots -- complete before lecture

Divergent channels branch out from the primary meridians and are mainly distributed on chest, abdomen, and _____ .

Sinew channels reflect the relationship between channels and body _____ .

Cutaneous regions are the surface _____ reflection of the 12 meridians.

Divergent channels strengthen the interior-exterior (_____) relationship between paired channels.

The 4-verb divergent-channel pattern is: Li, Ru, Chu, _____ .

The 12 divergent channels organize into _____ confluences (unions).

Next week (Week 8) covers the Eight _____ .

VOCABULARY PRE-LOAD -- DEFINE BEFORE CLASS

Purpose: Pre-loading key terms primes semantic memory so lecture content attaches to existing scaffolding.

PINYIN	ENGLISH	MY DEFINITION / CLINICAL NOTE
Jing Bie	Divergent Channel	
Jing Jin	Sinew Channel / Muscle Region	
Pi Bu	Cutaneous Region	
Liu He	Six Confluences / Six Unions	
Biao-Li	Exterior-Interior (paired-channel relationship)	
Li	"Separate" -- divergent channel leaves the primary pathway	
Ru	"Enter" -- divergent channel enters the body cavity/organs	
Chu	"Emerge" -- divergent channel re-emerges superficially	
He	"Converge/Join" -- divergent channel rejoins at the neck/head	
Jing Luo	General term: the whole channel system (jing = channels, luo = collaterals)	

The 6 Confluences (Liu He) -- fill in as Dr. Zhang confirms in lecture

YIN/YANG STAGE PAIRING	YANG CH.	YIN CH.
Foot Taiyang / Foot Shaoyin	BL	KI
Foot Shaoyang / Foot Jueyin	GB	LR
Foot Yangming / Foot Taiyin	ST	SP
Hand Taiyang / Hand Shaoyin	SI	HT
Hand Shaoyang / Hand Jueyin	SJ	PC
Hand Yangming / Hand Taiyin	LI	LU

D ANTICIPATORY QUESTIONS

* = high-challenge / high-yield

Purpose: Generating answers before instruction improves retention, even (especially) when you get it wrong.

Divergent Channels (Jing Bie)

Q1* THE 4-VERB PATTERN

Walk through Li -> Ru -> Chu -> He for a Yang-channel divergent branch. Where does it typically emerge from (hint: a joint)?

Q2* YIN VS YANG REJOINING

Where does a YIN channel's divergent branch rejoin -- back to its own primary channel, or somewhere else? What's different about the Yang pattern?

Q3 THE 6 CONFLUENCES

List all 6 Confluences (Liu He) pairs by primary channel abbreviation.

Q4 CLINICAL VALUE

What can divergent channels treat that primary-channel points alone might miss?

D ANTICIPATORY QUESTIONS (CONTINUED)

Sinew Channels (Jing Jin)

Q5* STRUCTURAL DIFFERENCE

What is the single biggest structural difference between a Sinew Channel and a primary Channel?

Q6 WHERE THEY CONCENTRATE

Sinew channels tend to gather/knot at which kind of body landmarks?

Q7 CLINICAL ASSOCIATION

What category of clinical conditions are Sinew Channels most associated with treating?

Cutaneous Regions (Pi Bu)

Q8* DIAGNOSTIC USE

How can changes in a Cutaneous Region (color, texture, temperature) be used diagnostically?

Q9 TRAP: COUNT

How many Cutaneous Regions are there, and do they map 1:1 onto the 12 primary channels?

Systems Comparison

Q10* LUO VS JING BIE

Collaterals (Luo) and Divergent Channels (Jing Bie) both 'branch off' a primary channel. What's the key functional difference between them?

Q11 WHY THIS ORDER

Why might Dr. Zhang's syllabus place Divergent/Sinew/Cutaneous (Week 7) BEFORE the Eight Extraordinary Vessels (Week 8), rather than after?

No Flinner transcript exists yet for this week's topic -- his parallel-section transcripts on file cover only Weeks 1-4. Nothing below is presented as verified Flinner-lecture content; it reuses corrections already verified from his Weeks 1-4 material, applied where directly relevant, and flags his general teaching emphasis as a study-approach suggestion, not a content claim.

Relevant already-verified Flinner corrections, applied to this week's material:

Don't confuse this week's NEW connections (Divergent-Channel Li/Ru/Chu/He links) with primary-channel Crossing/Meeting Points from earlier weeks -- different systems. Per Flinner's verified corrections: LU has only 1 true meeting point (shared with SP); LI has zero incoming meeting points; PC and HT both have zero crossing points. Keep these separate from this week's divergent-channel confluences.

Per Flinner's general emphasis across Weeks 1-4: prioritize the ROUTE pattern (the 4-verb Li/Ru/Chu/He logic and the 6 Confluences) over memorizing every point along each divergent branch -- most divergent channels reuse points you already know from the primary channels, so the new information this week is almost entirely structural, not point-level.

IQ INTER-QUIZ -- MID-STUDY PROBE

(checkpoint format)

Purpose: Spaced retrieval, checked every 4 items instead of at the end.
Mastery: $\geq 10/12$ across 3 separate sessions on different days.

Date: _____ Start: _____ End: _____ Session score: ____/12

CHECKPOINT 1 (items 1-4)

- 1 ACQ What does 'Jing Bie' mean, literally and functionally?
- 2 ACQ Name the 4 verbs describing a divergent channel's path, in order.
- 3 ACQ How many Confluences (Liu He) do the 12 divergent channels form?
- 4 MAINT What are the 3 circuits from the Master Map, and which weeks completed each? (Wks 1-6 review)

Self-check now (answers below) – score this block: ____/4 before moving on.

CHECKPOINT 2 (items 5-8)

- 5 ACQ Name the BL/KI confluence pairing by Yin/Yang stage name (e.g. Foot Taiyang/Foot Shaoyin).
- 6 ACQ What does a Sinew Channel lack that a primary channel has?
- 7 ACQ What is a Cutaneous Region, in one sentence?
- 8 MAINT Name all 8 confluent (opening) points you already know, with their vessel. (Wks 2-6 review)

Self-check now (answers below) – score this block: ____/4 before moving on.

CHECKPOINT 3 (items 9-12)

- 9 ACQ TRAP: Which week covers the Eight Extraordinary Vessels -- 7 or 8?
- 10 ACQ What's the key functional difference between Luo (Collaterals) and Jing Bie (Divergent Channels)?
- 11 MAINT State the Yin/Yang direction rule for Hand-Yang channels. (Wk1 Master Map review)
- 12 MAINT GB's slide claims 12 points/6 meridians for crossing points, but only how many are actually named? (Wk6 flag)

Self-check now (answers below) – score this block: ____/4 before moving on.

check each block right after attempting it

1. Jing Bie = 'channel divergence/separation' -- a deep branch that leaves the primary channel to strengthen ties between paired channels and the organs.
2. Li (separate) -> Ru (enter) -> Chu (emerge) -> He (converge).
3. 6 Confluences (Liu He) -- one per Yin/Yang paired-channel relationship.
4. Anterior Circuit (LU-LI-ST-SP, Wks 2-3), Posterior/Inner Circuit (HT-SI-BL-KI, Wks 4-5), Middle Circuit (PC-SJ-GB-LR, Wk 6).
5. Foot Taiyang / Foot Shaoyin (BL-KI).
6. A pertaining Zang/Fu organ -- Sinew Channels have no internal organ connection, only muscle/joint distribution.
7. The surface/skin zone where a primary channel's Qi and Blood reflect onto the body -- used for observation-based diagnosis.
8. LU7 (Ren Mai), KI6 (Yin Qiao), SI3 (Du Mai), BL62 (Yang Qiao), SP4 (Chong Mai), PC6 (Yin Wei), SJ5 (Yang Wei), GB41 (Dai Mai) -- all 8 confluent points, already complete as of Week 6.
9. Week 8 -- NOT Week 7. Week 7 is Divergent/Sinew/Cutaneous systems.
10. Divergent Channels reach deep into the torso/organs and reinforce interior-exterior pairing; Collaterals are more superficial, branching from Luo points to link paired channels directly without the same deep organ-level detour.
11. Hand -> Head.
12. 9 are actually named, though the slide states 12 points / 6 meridians -- flagged, unresolved pending Dr. Zhang confirmation.

Session 1: ____/12 Session 2: ____/12 Session 3: ____/12 Criterion met ($\geq 10/12 \times 3$): []

CLINICAL CASE -- APPLY IT BEFORE & AFTER

Purpose: Case reasoning bridges textbook concepts to clinical logic. Struggling before lecture is expected.

Case Vignette: Ms. Torres, 38 years old

Ms. Torres has chronic lateral hip and outer-thigh pain along the Gallbladder channel distribution that hasn't fully resolved with GB34 and local primary-channel points alone. There's a tight, ropy band palpable along the lateral thigh, worse with prolonged sitting, no clear organ-level GB symptoms (no bitter taste, no hypochondriac distention). Tongue and pulse unremarkable.

Pre-lecture: which supplementary system (not primary-channel points) might explain why local points alone aren't enough?

Post-lecture: revise your answer with what Dr. Zhang actually teaches about treating along a Sinew Channel.

F. After-Lecture Synthesis

Before I thought...	Now I know...	Still confused about...

Update Section A confidence scores now, then file this sheet with your Week 7 Study Guide once built.